DSM LAB · STUDY TOGETHER

The Unfolding Patient: role-play cards

Patients 81–90.

One person plays the patient; the other is the social worker doing an intake. **Print single-sided**, so the answer page never sits on the back of the patient card.

How to play

1. **Patient:** read the setting and your opening line aloud. Keep the rest of the card to yourself.
2. **Social worker:** interview. Ask about anything: onset and how long, sleep, substances and medications, medical history, safety, family, work or school, culture.
3. **Patient:** answer only what is asked, in your own words, using the "If asked about" table. Don't volunteer the items under "Only if asked".
4. **Social worker:** when ready (try 10 minutes), name a diagnosis and say what you'd do or check next.
5. Turn to the answer page together. Read the teaching point and the checkpoint question.

Each answer page follows its patient card. Keep it face down until the social worker commits to a diagnosis.

Scoring, if you want it

3 points for the right diagnosis, plus 1 for naming the rule-out (substance, medication or medical cause) before deciding, plus 1 for a safe, assess-first next step. Swap roles any time.

Playing it in the app

The same patients appear in DSM Lab → The Unfolding Patient, one clue at a time. If you play a card here first, the app version becomes a quick review.

These cases are original teaching material. Any resemblance to real people is coincidental.

Patient 81: The dogs under the bed

Setting: Delroy, 78, a retired tailor born in Jamaica, on a hospital urology unit two days after prostate surgery. The day nurse and the night nurse ask for a social work consult, and his wife, Hyacinth, joins at the bedside.

> *"Somebody let those dogs in again. Are you the manager? Put them out before my wife comes."*

If asked about…

When did this start?	Wife: The afternoon after the surgery he was sore but making jokes with me. That night they gave him the new pills, and by morning he was talking about his shop like it was 1985.
Where are you and what day is it?	This is the shop on King Street. No… a hotel? Is it Saturday? (Gets the day wrong and drifts off mid-answer.)
The things he sees	Two little brown dogs under the bed. Last night a man stood by the window measuring the curtains. Wife: He has never in his life seen things that weren't there.
Sleep	Night nurse: He dozes most of the day and is up and agitated at night. Wife: At home he sleeps like a stone. In 52 years I have never once seen him fight in his sleep.
Medicines	Day nurse: Before surgery, a blood pressure pill and a prostate pill for years. After surgery, acetaminophen for pain; he turned down the stronger pain pills. Oxybutynin for bladder spasms and diphenhydramine for sleep were both started two nights ago.
Alcohol and drugs	Wife: A small glass of rum at Christmas and weddings. Nothing daily. He has never smoked or used drugs.
Medical history	Day nurse: High blood pressure and an enlarged prostate. No strokes or seizures. His temperature and his blood tests this morning were normal.
Daily life before surgery	Wife: He handles all our accounts, drives us to church and still does alterations for the neighbors. He threads a needle without glasses.
Walking and movement	Wife: He walks two miles every morning. No shaking, no shuffling, no falls.
Thoughts of suicide or harming others	No, man, I want to go home. Night nurse: He is not trying to hurt himself, but he could fall or pull out his catheter at night, so we have someone sitting with him.
Family history	Wife: His mother lived to 94 and was sharp to the end. His brother had a stroke at 80.
Culture and context	Wife: We came from Kingston in 1979. He is a very proper man, and he would be ashamed to know he shouted at the nurses. Our pastor is coming this evening to pray with him.

Only if asked (don’t volunteer)

- Oxybutynin and diphenhydramine were started two nights ago
- The confusion began that same night
- He balanced the household accounts a week ago
- No earlier visions, sleep thrashing or walking problems

Patient 81: answer and debrief

DIAGNOSIS

Delirium

Small animals he can see clearly, confusion that comes and goes, and thrashing at night make Lewy body disease a fair early guess. Clue 4 flips it: two strongly anticholinergic medicines, oxybutynin and diphenhydramine, were started two nights ago, and the confusion began that same night. That is the detail that separates the two: delirium starts over hours to days with a medical cause, while Lewy body disease unfolds over months, with dream enactment and parkinsonism that come before any hospital stay. His attention is badly impaired, and his wife confirms he was sharp a week ago with no earlier visions, sleep thrashing or walking problems. Exam lesson: delirium is a medical emergency, so ask for an urgent medical review; expect the team to stop or replace anticholinergic drugs, which the Beers Criteria advise avoiding in older adults. Reassess his thinking after the delirium clears.

What had to be ruled out

- Neurocognitive disorder with Lewy bodies: onset in one night after new medicines, not over months; no earlier visual hallucinations, dream enactment or parkinsonism
- Major neurocognitive disorder: managing the household accounts a week ago; deficits seen only during a delirium cannot support the diagnosis
- Psychotic disorder due to another medical condition or a primary psychotic disorder: the hallucinations come with disturbed attention and awareness, which is delirium
- Alcohol withdrawal: drinks only a small glass of rum on holidays

Checkpoint: What would you ask or check NEXT?

A. Tell the medical team this is a sudden change and ask for an urgent review of his new medicines and other causes of delirium **(best)**

B. Ask the team to give him an antipsychotic to stop the visions and the night agitation

C. Recommend a full outpatient memory evaluation after he goes home

New confusion with fluctuating attention in an older adult after surgery is delirium until proven otherwise, and delirium is a medical emergency. The cause has to be found, and new medicines are a leading suspect. B treats a symptom without finding the cause, and if this were Lewy body disease an antipsychotic could cause a severe reaction. C delays urgent care, and memory testing during a delirium would not show his true baseline.

The case in six clues (as the app reveals it)

1. Day nurse: This morning he thought he was in his old tailor shop and asked me to pass him the chalk. An hour later he knew exactly where he was and asked about lunch.
2. There are two little brown dogs under the bed. I can see their ears. Tell the nurse to put them out before somebody trips.
3. Night nurse: Around two in the morning he was shouting and swinging his arms, like he was fighting someone in a dream, and he pulled at his catheter. By breakfast he was calm again.
4. Day nurse: After surgery he had bladder spasms, so two nights ago the team started oxybutynin, and the night doctor added diphenhydramine for sleep. The mix-ups started that same night. His mouth is bone dry.
5. Day nurse: When I ask him to name the months backward, he gets to October and drifts off talking about a suit he's sewing. He can't keep his attention on me for more than a few seconds.
6. Wife: Last week he balanced our accounts to the penny and argued with me over a bank fee. He has never seen things that weren't there or fought in his sleep, and he walks two miles every morning. None of this started until the night after the operation.

What a skilled clinician weighs after each clue

1. Delirium, Major neurocognitive disorder, Neurocognitive disorder with Lewy bodies
2. Delirium, Neurocognitive disorder with Lewy bodies, Psychotic disorder due to another medical condition
3. Neurocognitive disorder with Lewy bodies, Delirium, Major neurocognitive disorder
4. Delirium, Neurocognitive disorder with Lewy bodies
5. Delirium, Major neurocognitive disorder
6. Delirium

Sources: DSM-5-TR (APA, 2022), Neurocognitive Disorders: Delirium (substance/medication-induced); American Geriatrics Society Beers Criteria Update Expert Panel (2023), Journal of the American Geriatrics Society 71(7):2052-2081 (strongly anticholinergic drugs)

Patient 82: The man by the curtain

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Evelyn, 72, a retired school librarian, in a hospital emergency department late at night. Her daughter, Joy, brought her, and the ED nurse is at the bedside with the social worker.

> *"Joy, there's a gentleman by the curtain. Ask him if he'd like to sit down."*

If asked about…

When did this start?	Daughter: About nine months ago. At first she'd lose track for an afternoon and be fine the next day. Now a bad spell comes every week or two.
The visions	A man in a gray uniform, very polite. Sometimes children sitting on the stairs. They don't speak. Some days I know they can't be real, and some days I'm not sure.
Sleep	Daughter: My father used to say she talked and kicked in her sleep for at least ten years. Now she naps twice a day and still seems drowsy.
Movement	Daughter: She started shuffling about four months ago, before the pill her doctor tried. Her right hand trembles a little when it's resting in her lap.
Medicines	Daughter: A cholesterol pill and eye drops, for years. No sleep aids, not even over-the-counter ones. The antipsychotic two months ago made her stiff and sleepy, and she fell twice; he stopped it after three days. The extra stiffness wore off within a week, but the shuffle she had before is still there.
Alcohol and drugs	Daughter: She has never been a drinker. A small glass of sherry on holidays. No drugs.
Medical history	Daughter: Two ER visits in nine months, both with normal tests and scans. No strokes, no head injuries, no fever tonight. Her eyesight is good with glasses.
Daily functioning	Daughter: Five months ago I took over her pills and checkbook after she paid the water bill three times and missed doses. She still dresses and bathes herself and walks her dog with me.
Mood	I'm not sad, dear. I still listen to my mystery books every night. Daughter: She gets frustrated on the foggy days, but she's not depressed.
Thoughts of suicide or harming others	Goodness, no. Daughter: She has never talked about dying or hurting anyone. My worry is falls.
Family history	Daughter: Nobody with memory trouble that we know of. Her parents both died in their seventies of heart trouble.
Culture and context	Daughter: She ran the town library for thirty years, and everyone knows her. She's private and proud. She hid the bad days from her book club for months and only agreed to come tonight because she couldn't find her own front door.

Only if asked (don’t volunteer)

- Spells like this have recurred for nine months, with two earlier normal ER workups
- About ten years of talking and kicking in her sleep
- Severe stiffness, falls and drowsiness after a low-dose antipsychotic
- Shuffling began four months ago, after the cognitive decline and before the antipsychotic
- Daughter manages her pills and checkbook

Patient 82: answer and debrief

DIAGNOSIS

Neurocognitive disorder with Lewy bodies

An older woman who suddenly doesn't know her daughter and sees a detailed figure by the curtain looks like delirium, and in the ED that must be checked first. Clue 4 flips it: these spells have come and gone for nine months, each time with normal tests, and she has declined between them. That is the detail that separates the two: Lewy body fluctuations recur over months with a negative workup, while delirium is a new change over hours to days with a medical cause. Recurrent well-formed visual hallucinations, years of acting out dreams, shuffling and a masklike face that began after the cognitive decline, and a severe reaction to a low-dose antipsychotic all point to Lewy bodies. Her daughter manages her pills and money, so it is a major NCD. Exam lesson: make sure her chart and family know about the antipsychotic sensitivity, and support the caregiver, who is also watching for falls.

What had to be ruled out

- Delirium: normal urine, blood work and head scan; no medicine change; the same spells have recurred for nine months rather than a new acute change
- Major neurocognitive disorder due to Alzheimer's disease: early fluctuations, visual hallucinations and dream enactment fit Lewy bodies better
- Major neurocognitive disorder due to Parkinson's disease: the shuffling began months after the cognitive decline, not years before it
- Medication-induced parkinsonism: the shuffling began before the antipsychotic trial and has stayed since it was stopped
- Psychotic disorder due to another medical condition or schizophrenia: the hallucinations are part of the cognitive syndrome, with first onset at 71 and a negative workup

Checkpoint: What would you ask or check NEXT?

A. Ask the ED doctor for a dose of an antipsychotic so she can rest tonight

B. Ask her daughter whether spells like this have happened before and how she has been between them **(best)**

C. Tell her daughter this is dementia and help her start looking at memory care homes

A normal workup tonight does not settle the question; the course does. Delirium is a new change from baseline, while a neurocognitive disorder has a history, so the daughter's account of past spells and the baseline between them comes next. A risks a severe reaction, because some older adults with visual hallucinations have Lewy body disease, and sedation is not assessment. C names a diagnosis and plans placement before the history is known.

The case in six clues (as the app reveals it)

1. Daughter: This afternoon she didn't know who I was for almost an hour. She kept asking when my father would be home from work, and he died six years ago.
2. Nurse: She's describing a man in a gray uniform standing by the curtain, down to the buttons on his coat. She's calm about it and asked me to offer him a chair.
3. Nurse: Her urine, blood work and head scan tonight are all normal, and her daughter says no medicines have been started or changed in the last month. Right now she's following the conversation fine.
4. Daughter: This isn't new. She's had spells like this on and off for about nine months, and we've been to this ER twice before with normal tests. Between spells she's foggier than she used to be, and I took over her pills and checkbook five months ago.
5. Daughter: For about ten years she's talked and kicked in her sleep, like she's acting out her dreams. In the last four months she's started to shuffle, and her face doesn't show much expression anymore.
6. Daughter: Two months ago her doctor tried a low dose of an antipsychotic pill for the visions. Within three days she was stiff as a board, fell twice and slept most of the day, so he stopped it.

What a skilled clinician weighs after each clue

1. Delirium, Major neurocognitive disorder, Neurocognitive disorder with Lewy bodies
2. Delirium, Neurocognitive disorder with Lewy bodies, Psychotic disorder due to another medical condition
3. Delirium, Neurocognitive disorder with Lewy bodies, Major neurocognitive disorder
4. Neurocognitive disorder with Lewy bodies, Major neurocognitive disorder
5. Neurocognitive disorder with Lewy bodies, Major neurocognitive disorder
6. Neurocognitive disorder with Lewy bodies

Sources: DSM-5-TR (APA, 2022), Neurocognitive Disorders: Major or Mild Neurocognitive Disorder With Lewy Bodies; McKeith et al. (2017), DLB Consortium diagnostic criteria, Neurology 89:88-100

Patient 83: The baker's books

Setting: Augusto, 73, who ran a neighborhood bakery for 41 years, at a memory clinic on his primary care doctor's referral. His niece, Bianca, came with him, and the clinic's testing technician shares a note.

> *"My niece thinks I'm losing my mind. Maybe she's right. My memory is finished."*

If asked about…

When did this start?	Niece: He closed the bakery four months ago when the lease wasn't renewed. Two weeks later he stopped going out. The memory talk started about two months ago.
Memory	I forget everything. Don't ask me, I don't know. (If gently pressed, he can say what he ate for breakfast and that his grandnephew scored a goal on Saturday.)
Mood	Heavy. Sad all day. Forty-one years I got up at three to make bread. Now what am I for?
Sleep and energy	I fall asleep fine, but I'm awake at four every morning, thinking. Then I'm tired all day. I nap in the chair.
Appetite and weight	Nothing tastes good. Niece: His pants are loose; I think he's lost about ten pounds.
Interests	I stopped going to Mass. I don't go to the card game. I don't even cook for myself, and I was a baker.
Bills and daily tasks	I don't open the mail. Why bother? Niece: When I sat with him, he wrote every check correctly and remembered which ones were late. He still showers, dresses and takes his pills on his own.
Alcohol, drugs, medications	A glass of wine with dinner, less now. A blood pressure pill for twenty years. No sleeping pills.
Medical history	Niece: His doctor checked his blood, thyroid and B12, all normal. No strokes, no head injuries. His hearing is fine.
Thoughts of suicide or harming others	Sometimes I think it would be easier not to wake up. But I would never do anything. My mother, God rest her, would never forgive me. I have no plan. There are no guns in my house.
Family history	My father had a 'nervous sadness' and stayed in bed for months after his business failed. Nobody in my family lost their memory.
Culture and context	I came from Sao Paulo at 25 with my brother. The bakery was our family's pride; the whole neighborhood came. Niece: My mother wanted him to talk to the priest instead of a doctor. He agreed to both.

Only if asked (don’t volunteer)

- The bakery closed four months ago, and loss of interest came before the memory complaints
- He wrote the bills correctly when his niece sat with him
- Passive wish not to wake up, with no plan or intent and no guns at home
- His father had a long depression after a business loss

Patient 83: answer and debrief

DIAGNOSIS

Major depressive disorder

Memory complaints and missed bills in a 73-year-old make major NCD the first guess. Clue 3 hints and clue 4 flips it: the trouble has a clear start, right after he lost his bakery four months ago, and loss of interest and withdrawal came first, with memory complaints weeks later. That is the detail that separates the two: depression has a datable onset with mood symptoms first, while a neurocognitive disorder creeps in over months to years. He complains loudly about his memory, answers 'I don't know' quickly and improves with encouragement, the picture once called pseudodementia; people with a neurocognitive disorder more often try hard and play down their deficits. He has at least six depressive symptoms for months, so this is MDD, not adjustment disorder, and he can still do his bills when someone sits with him. Exam lesson: treat the depression and retest; late-life depression can also come before a later dementia, so follow up. His wish not to wake up needs a full suicide risk assessment: older men have among the highest suicide rates in the US.

What had to be ruled out

- Major neurocognitive disorder: datable onset after a loss, mood symptoms first, poor effort with better recall on hints, and he writes the bills correctly with company
- Mild neurocognitive disorder: no decline shown with full effort; the deficits follow the depression
- Adjustment disorder: full major depressive episode criteria are met, so MDD is diagnosed
- Depressive disorder due to another medical condition: normal blood tests, thyroid and B12; no new medicines
- Prolonged grief disorder: the loss is his business, not a death

Checkpoint: What would you ask or check NEXT?

A. Tell his niece the testing shows dementia and discuss a move to assisted living

B. Repeat the full memory test today and insist that he finish every section

C. Ask about his mood, sleep, appetite and recent losses, and whether the memory trouble started before or after any low mood **(best)**

Quick 'I don't know' answers, giving up and better recall with hints suggest depression rather than a neurocognitive disorder, so a depression screen and a timeline come next. A names dementia and plans a move on testing that reflects poor effort. B pushes a poorly engaged client; the effort pattern is itself the finding, and forcing it would not give a valid score.

The case in six clues (as the app reveals it)

1. My memory is gone. I forget names, I lose my glasses, I can't remember anything. I'm useless now.
2. Niece: He forgot his own sister's birthday, and he's stopped paying his bills on time. Last month the gas company sent a final notice.
3. Technician's note: On the memory test he said 'I don't know' to many questions within seconds and asked to stop twice. With encouragement he got several right, and with hints he recalled most of the word list.
4. Niece: The landlord didn't renew his lease, and he closed the bakery four months ago. Within two weeks he stopped going to Mass and seeing his friends, and he says nothing interests him; the memory complaints only started about two months ago. Before that he did his own books to the penny.
5. I feel heavy and sad all day. I wake at four every morning and lie there thinking I'm a burden to my family. Nothing tastes good, and I'm tired all the time.
6. Niece: His blood tests, thyroid and B12 were normal, and no medicines have changed. When I sat with him, he wrote out the bills correctly; he just doesn't open the mail anymore. Last week he told me it would be easier not to wake up, but he says he would never do anything to himself.

What a skilled clinician weighs after each clue

1. Major neurocognitive disorder, Mild neurocognitive disorder, Major depressive disorder
2. Major neurocognitive disorder, Major depressive disorder, Mild neurocognitive disorder
3. Major depressive disorder, Major neurocognitive disorder
4. Major depressive disorder, Major neurocognitive disorder, Adjustment disorder
5. Major depressive disorder, Major neurocognitive disorder
6. Major depressive disorder

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Major Depressive Disorder; Neurocognitive Disorders: Major Neurocognitive Disorder (differential diagnosis); Yesavage et al. (1982-1983), Geriatric Depression Scale, Journal of Psychiatric Research 17(1):37-49

Patient 84: The coach with the flu

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Bridget, 46, a high school softball coach, at an urgent care clinic. Her wife, Marisol, drove her, and the urgent care nurse asks the behavioral health clinician to see her.

> *"I think I'm losing it. I'm sweating, I'm shaking, and I can't sit still. Maybe it's the flu, maybe it's my nerves."*

If asked about…

When did this start?	Last night. I took my last pill yesterday morning, and by bedtime I was restless. I didn't sleep at all.
Medicines	Oxycodone, one pill four times a day since the surgery seven weeks ago, exactly as written. The office said no more refills and to use ibuprofen. Nobody mentioned tapering. No sleeping pills, no anxiety pills.
Physical symptoms	Everything aches. Chills, runny nose, my eyes keep watering, I can't stop yawning, and I've had diarrhea four times since this morning. I feel sick to my stomach.
Sleep	None last night. I tossed and turned and kept kicking my legs.
Alcohol and drugs	A beer after a tournament win. That's it. I've never used drugs. Wife: That's true.
Medical history	Healthy except my back. No thyroid or heart problems. The nurse said my temperature is normal.
Worry and anxiety history	I worry like any coach, about lineups and parents. Wife: She's steady. She sleeps like a rock normally and has never had panic attacks.
Pain	My back is sore, but it's better than before surgery. I'm scared it'll get worse without the pills.
Thoughts of suicide or harming others	No, I don't want to hurt myself or anyone. I just want this to stop. A parent on the team said she could get me some pills, and honestly, at three this morning I thought about it.
Family history	My dad drank too much. That's why I was so careful to take the pills exactly as written.
Work and functioning	I had to cancel practice today. I can't stand on a field like this.
Culture and context	Coaches are supposed to be tough. My players don't know how rough the surgery was. My wife made me come in.

Only if asked (don’t volunteer)

- Took her last oxycodone yesterday morning after seven weeks of daily use
- Always took it exactly as prescribed
- A team parent offered her pills, and she considered it
- Her father drank heavily

Patient 84: answer and debrief

DIAGNOSIS

Opioid withdrawal

Restlessness, a racing heart and no sleep look like anxiety, and a self-described worrier with aches and a runny nose in flu season seems to have an easy explanation. Clue 4 flips it: she stopped seven weeks of daily oxycodone about a day ago. That is the detail that separates the two: the symptoms began right after stopping an opioid. Aches, runny nose, tearing, yawning, goosebumps, wide pupils, diarrhea and insomnia are the opioid withdrawal picture, her normal temperature argues against flu, and short-acting opioids usually bring it on within about a day. DSM-5-TR does not count tolerance or withdrawal toward opioid use disorder when the opioid is taken as prescribed under medical care, and nothing else here (extra doses, early refills, other sources) points to OUD, so this is opioid withdrawal without a use disorder. Exam lesson: opioid withdrawal is rarely life-threatening for a healthy adult but it is miserable. Help her reach her surgeon or primary doctor today for a slow taper or symptom treatment, because guidelines advise against stopping opioids abruptly after weeks of daily use, and someone in withdrawal may be tempted by street pills that can contain fentanyl, so offer naloxone.

What had to be ruled out

- Generalized anxiety disorder: her usual worry is ordinary and never disturbed her sleep; these symptoms began about a day after stopping the opioid
- Panic disorder: no discrete, unexpected attacks; steady restlessness tied to stopping the medicine
- A viral illness such as flu: normal temperature; tearing, yawning, goosebumps and wide pupils fit withdrawal
- Opioid use disorder: not established; taken as prescribed with no extra doses, early refills or other sources, and tolerance and withdrawal from prescribed use do not count
- Alcohol or sedative withdrawal and stimulant intoxication: rare beer, no sedatives or stimulants

Checkpoint: What would you ask or check NEXT?

A. Teach her slow breathing and relaxation skills and refer her for outpatient therapy for anxiety

B. Ask about any medicines, alcohol or drugs she has recently started, changed or stopped **(best)**

C. Suggest she ask the doctor for a benzodiazepine to calm her nerves

Sudden anxiety with aches, a runny nose and diarrhea calls for asking about substances and medications before naming an anxiety disorder, and that includes medicines stopped as well as started. A treats a disorder that is not established and misses a physical cause. C could mask the cause, and benzodiazepines taken with opioids carry an FDA boxed warning for dangerous sedation and slowed breathing.

The case in six clues (as the app reveals it)

1. I feel like I'm crawling out of my skin. I can't sit still, my heart is racing, and I didn't sleep at all last night. I think I'm having a breakdown.
2. My whole body aches, I've got chills, my nose won't stop running, and I've had diarrhea since this morning. Half my team has had the flu.
3. I've always been a worrier, and for weeks I've been stressed about the season and my back.
4. Seven weeks ago I had back surgery, and I've taken the oxycodone exactly as prescribed, four a day. The surgeon's office said no more refills, so I took my last one yesterday morning.
5. Nurse: Her temperature is normal. Her pupils are wide, she has goosebumps on her arms, and she has yawned and wiped her eyes all through the visit.
6. Wife: She never took extra or ran out early, and she never looked for pills anywhere else. Before the surgery she'd never taken anything stronger than ibuprofen. She's a normal worrier, and it never kept her up at night before this.

What a skilled clinician weighs after each clue

1. Generalized anxiety disorder, Panic disorder, Anxiety disorder due to another medical condition
2. Anxiety disorder due to another medical condition, Generalized anxiety disorder, Opioid withdrawal
3. Generalized anxiety disorder, Opioid withdrawal, Panic disorder
4. Opioid withdrawal, Opioid use disorder
5. Opioid withdrawal, Opioid use disorder
6. Opioid withdrawal

Sources: DSM-5-TR (APA, 2022), Substance-Related and Addictive Disorders: Opioid Withdrawal; Opioid Use Disorder (prescribed-use note); Dowell et al. (2022), CDC Clinical Practice Guideline for Prescribing Opioids for Pain, MMWR Recommendations and Reports 71(3):1-95 (tapering; avoiding abrupt discontinuation); SAMHSA TIP 63, Medications for Opioid Use Disorder (2021), Overdose risk and naloxone

Patient 85: The editor on deadline

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Kieran, 33, a freelance video editor, at a primary care visit he booked to ask for an ADHD evaluation. His mother joins by phone for part of the visit.

> *"I just need someone to sign off on ADHD meds so I can do my job."*

If asked about…

When did the focus problems start?	Maybe two years ago, around when I went freelance. Before that I was fine, I guess.
School and childhood	School was easy. Mother: Straight A's, color-coded binders, never lost a permission slip. Teachers loved him.
How much do you use, and where do you get it?	At first half a pill for a deadline, from a roommate. Now six to eight a day, from a guy I met online. They look like the real thing.
What happens when you run out?	I crash. I sleep fourteen hours, eat everything, feel flat and hopeless, and can't string two thoughts together.
Trying to cut back	Twice. Once I made it a week, then I had a big job and caved. The other time lasted two days.
Sleep	Three or four hours a night when I'm using. Fourteen when I crash.
Alcohol and other drugs	A couple of beers on weekends. No other drugs.
Medical history	I've lost about fifteen pounds this year. Sometimes my heart pounds. No seizures or heart problems that I know of. No medicines.
Mood and thoughts of suicide	On crash days I feel pretty hopeless, but I've never thought about killing myself and I've never hurt myself. No thoughts of hurting anyone.
Money and relationships	I was short on rent twice. My girlfriend moved out three months ago. She said she was done watching me chase pills.
Family history	My uncle had a drinking problem. Nobody has ADHD that I know of.
Culture and context	In my world everybody's on something to hit deadlines. Freelance means if you're slow, you don't eat. Asking a doctor felt more legit than buying from a guy.

Only if asked (don’t volunteer)

- His focus was fine until he started pills two years ago
- Buys pills from a dealer and now takes six to eight a day
- Two failed attempts to stop
- Mother reports no childhood attention problems

Patient 85: answer and debrief

DIAGNOSIS

Stimulant use disorder

Poor focus, lost items and interrupting look like ADHD, and he says stimulants help. Clue 4 flips it: his mother describes an organized, focused child, with nothing like this until the last couple of years. That is the detail that separates the two: ADHD requires several symptoms before age 12, and his began in adulthood, with the pills. Clue 5 shows rising doses (tolerance) and crashes when he runs out (withdrawal). With two failed attempts to stop, missed deadlines and use that cost him his relationship, he meets at least four of the eleven criteria. Exam lesson: a good response to a stimulant does not prove ADHD; check onset with collateral and screen for misuse before anyone prescribes. Pills bought on the street may be counterfeit and can contain methamphetamine or fentanyl, so offer naloxone and harm-reduction information, and check mood and suicide risk during crashes.

What had to be ruled out

- ADHD: no symptoms before age 12 by his mother's account and report cards; attention problems began with the pills
- Major depressive disorder: low mood and oversleeping come on crash days, tied to running out of stimulants
- Stimulant intoxication: he describes a two-year pattern, not a single acute episode
- Generalized anxiety disorder: no chronic worry before or apart from the drug use

Checkpoint: What would you ask or check NEXT?

A. Ask how often and how much he uses Adderall and where he gets it, and get childhood history from someone who knew him then **(best)**

B. Start a trial of a long-acting stimulant, since the Adderall clearly helps

C. Give him an adult ADHD rating scale and diagnose ADHD if he scores above the cutoff

Stimulants sharpen focus in almost anyone, so a good response proves nothing. The next step is a substance history and collateral about childhood, because ADHD requires several symptoms before age 12. B could feed a use disorder. C relies on self-report alone, which cannot show childhood onset or rule out the effects of a drug.

The case in six clues (as the app reveals it)

1. I can't focus on anything. I open a project, drift to my phone, and three hours are gone. I'm missing deadlines.
2. An online quiz says I have ADHD for sure. Adderall is the only thing that helps me focus; I just need my own prescription.
3. My desk is chaos, I lose my keys and chargers, and I interrupt clients on calls.
4. Mother: He was the most organized child I knew. Every report card through high school said 'focused' and 'a pleasure to teach.' I never saw any of this until the last couple of years.
5. Honestly, my focus was fine until two years ago, when I took half a pill to pull an all-nighter. Now I buy them from a guy, six to eight a day, because two stopped doing anything. When I run out I sleep fourteen hours, eat everything in sight, feel hopeless and can't think at all.
6. I've tried to stop twice, once for a whole week, and I caved both times. I've spent rent money on them, and my girlfriend moved out over it.

What a skilled clinician weighs after each clue

1. ADHD, Major depressive disorder, Generalized anxiety disorder
2. ADHD, Stimulant use disorder
3. ADHD, Stimulant use disorder
4. Stimulant use disorder, Major depressive disorder
5. Stimulant use disorder
6. Stimulant use disorder

Sources: DSM-5-TR (APA, 2022), Substance-Related and Addictive Disorders: Stimulant Use Disorder; Neurodevelopmental Disorders: ADHD (onset before age 12); SAMHSA TIP 33 (updated 2021), PEP21-02-01-004, Treatment for Stimulant Use Disorders

Patient 86: The next big thing

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Celeste, 46, a regional sales director for a medical supply company, at a private outpatient practice. Her husband, Gordon, asked her to come and joins for part of the visit.

> *"I'm only here because Gordon insisted. I'll be honest: I don't think I'm the problem."*

If asked about…

Why now?	My team went to HR, and HR put me on a performance plan. It's insulting.
The new company	In two years I'll be bigger than my CEO. Husband: She's talked about launching her own firm since graduate school. This time she put money down.
Sleep	Seven hours, eleven to six. Always. I don't need less, and I don't want less.
Energy and speech	Same as ever. Husband: She doesn't talk fast. She just doesn't let anyone else talk.
Any times you felt very different from usual?	No. I'm consistent. That's my strength. Husband: No highs and no crashes in twenty years.
Criticism	My manager wrote that I need to work on listening. I sat in my car shaking, then emailed the CEO. I deserve better than a review written by someone who's threatened by me.
Friends and relationships	Most people are intimidated by me. Friends drift off. Husband: She expects people to come to her, and they stop coming.
Alcohol, drugs, medications	Wine with clients. No drugs. No medicines except an allergy spray.
Medical history	My checkup four months ago was perfect, thyroid included.
Thoughts of suicide or harming others	Hurt myself? Absolutely not. And I'm not going to hurt anybody; I'm going to outperform them. Husband: She has never been violent.
Family history	My father was a surgeon who expected perfection. Nobody had mood problems that I know of.
Culture and context	I'm the first woman to run this region. A man who acted like me would get a bonus, not HR. Husband: Maybe some of that is true, but she's the same at home and with her family.

Only if asked (don’t volunteer)

- The pattern has been the same for twenty years
- Sleeps seven hours every night
- Emailed the CEO after a mild criticism
- Complained that hospital visits to her mother-in-law cut into her schedule

Patient 86: answer and debrief

DIAGNOSIS

Narcissistic personality disorder

Grand plans, an office lease and a new car look like mania, so bipolar I leads early. Clue 4 flips it: her husband says she has been the same for twenty years, sleeps seven hours, speaks at a normal pace and has never had a period when she was suddenly different. That is the detail that separates the two: mania is an episode, a change from baseline lasting at least a week with more energy and less need for sleep, while narcissistic grandiosity is a steady trait. Entitlement, need for admiration, envy, lack of empathy, and a collapse into hurt and rage after mild criticism (fragile self-esteem under the grandiosity) fit narcissistic personality disorder. Exam lesson: ask 'Is this different from how she usually is?' before thinking about mood episodes. Sharp drops in self-esteem after a setback can raise suicide risk in narcissistic personality disorder, so ask about it, as with any client.

What had to be ruled out

- Bipolar I disorder: no distinct episode; sleep, energy and speech are her usual; the pattern is twenty years long
- Bipolar II disorder: no hypomanic episodes either
- Borderline personality disorder: no self-harm, emptiness or unstable identity; anger follows threats to her status, not fear of abandonment
- Antisocial personality disorder: she seeks admiration, not gain through deceit, and has no history of conduct problems
- Histrionic personality disorder: she wants admiration for superiority, not any attention at all

Checkpoint: What would you ask or check NEXT?

A. Refer her to a psychiatrist for a mood stabilizer to treat the manic episode

B. Confront her about the spending so she sees its impact on the marriage

C. Ask about her recent sleep, energy and pace of speech, and whether this is a change from how she usually is **(best)**

Grand plans and sudden spending raise the question of mania, and mania is defined by a distinct period that differs from the person's usual self, with more energy and less need for sleep. Asking about baseline and sleep comes first. A treats an episode that has not been established. B confronts before assessing, which harms the alliance and does not answer the diagnostic question.

The case in six clues (as the app reveals it)

1. Husband: She says she's going to quit and start her own company, and that in two years she'll be bigger than her CEO.
2. Husband: Last week she put a deposit on an office downtown and ordered a new car, without a word to me.
3. My team filed a complaint because I told them the truth: they're mediocre. People at my level shouldn't have to explain themselves to HR.
4. Husband: Honestly, she's been like this since we met in graduate school twenty years ago. She sleeps seven hours like clockwork, she talks at a normal pace, and there has never been a stretch when she suddenly seemed different. The office and the car are just bigger versions of what she always does.
5. When my manager wrote that I 'need to work on listening,' I sat in my car for an hour, shaking. Then I emailed the CEO to say she was about to lose the best person she has.
6. Husband: When my mother was in the hospital about a year ago, Celeste complained that the visits were cutting into her schedule. Her friendships never last, she expects special treatment everywhere, and she resents anyone who gets credit. She has never hurt herself, and her mood stays steady unless someone crosses her.

What a skilled clinician weighs after each clue

1. Bipolar I disorder, Narcissistic personality disorder, Bipolar II disorder
2. Bipolar I disorder, Narcissistic personality disorder, Bipolar II disorder
3. Narcissistic personality disorder, Bipolar I disorder
4. Narcissistic personality disorder, Histrionic personality disorder
5. Narcissistic personality disorder, Borderline personality disorder
6. Narcissistic personality disorder

Sources: DSM-5-TR (APA, 2022), Personality Disorders: Narcissistic Personality Disorder; DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Bipolar I Disorder (manic episode)

Patient 87: The new roommate

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Luis, 37, a payroll clerk, at a community counseling center. His older sister, Pilar, came with him and joins for part of the visit.

> *"Pilar says I need to learn to stand on my own. I don't even know where to start."*

If asked about…

What happened?	My girlfriend left three weeks ago. She said she was tired of being my mother. Two weeks later I moved in with Daniela.
The new relationship	She's nice. She tells me what to do, which helps. I moved in after we'd met maybe six times.
Decisions	I text Pilar about lunch. I can't pick a shirt in a store without asking someone. I've never made a big purchase on my own.
Disagreeing with people	I never say no. If someone's upset with me, I fix it, even if I have to say I'm wrong when I'm not. I'd rather lose the argument than lose the person.
Being alone	The two weeks I was alone, I slept at my sister's every night. The empty apartment felt like I'd fall apart.
Mood, sleep and appetite	I cried every day the first week. Since I moved in with Daniela I sleep fine and eat fine. I'm okay now.
Anger, self-harm and suicide	I don't really get angry. I've never hurt myself, and I've never thought about suicide, not even after she left. I'd never hurt anyone.
Work	I'm good at payroll as long as my supervisor tells me exactly what to do. I turned down a team lead job because I'd have to make calls on my own.
History of the pattern	Sister: Our mother did everything for him. In high school his girlfriend planned his homework schedule. Then the next one, then the next. He's never lived alone.
Alcohol, drugs, medications	A beer at family parties. No drugs. No medicines.
Medical history	I'm healthy. My checkup last year was fine. No head injuries.
Culture and family	In our family we're close; everybody helps everybody. Sister: Sure, but the rest of us still choose our own lunch. Even our mother says he needs to grow up.

Only if asked (don’t volunteer)

- Moved in with a near stranger two weeks after the breakup
- Turned down a promotion because he would have to decide things alone
- Has gone from caretaker to caretaker since age sixteen
- Slept at his sister's every night while single

Patient 87: answer and debrief

DIAGNOSIS

Dependent personality disorder

A breakup, panic at being alone and a rushed move-in with a near stranger suggest borderline personality disorder. Clue 5 flips it: he meets the threat of being left with appeasement, agreeing even when he thinks she's wrong, with no anger, self-harm or unstable mood. That is the detail that separates the two: borderline reacts to abandonment with rage and emptiness, dependent with submission and an urgent search for a new caretaker. He needs others to make everyday choices, cannot start things on his own, cannot disagree, and has gone from caretaker to caretaker since his teens. He is outgoing and seeks relationships, which rules out avoidant personality disorder, where people hold back for fear of rejection. Exam lesson: weigh cultural norms about family closeness before calling reliance a disorder; here his sister, raised in the same family, says his reliance goes far beyond the family's norms.

What had to be ruled out

- Borderline personality disorder: no anger outbursts, self-harm, emptiness or identity disturbance; he meets abandonment with appeasement
- Avoidant personality disorder: socially at ease; he seeks relationships rather than avoiding them
- Major depressive disorder or adjustment disorder: his mood settled as soon as a new caretaker was in place; sleep and appetite are normal
- Cultural norm: his family values closeness, but his sister says his reliance goes far beyond the family's norms

Checkpoint: What would you ask or check NEXT?

A. Help him list the reasons he should move out of the new relationship right away

B. Ask about suicidal thoughts and self-harm, then explore how he has handled past breakups and everyday decisions **(best)**

C. Refer him to a DBT program for borderline personality disorder

A recent breakup with daily crying and fear of being alone calls for a safety check first, then a history of the pattern across relationships, which is what tells the personality disorders apart. A is directive and imposes the clinician's choice on him. C names a disorder before the pattern has been assessed.

The case in six clues (as the app reveals it)

1. My girlfriend of eight years left me three weeks ago. I cried every day the first week, and I don't know how to be on my own.
2. Two weeks after she left, I moved in with a woman I met at the gym. I barely know her, but I couldn't stand being alone.
3. Sister: This isn't new. Since he was sixteen, he's gone straight from one person who takes care of him to the next, with hardly a week alone.
4. Sister: His girlfriend picked his clothes, his meals, even which shifts to take. Now he texts me to ask what to order for lunch, and he asked his new roommate whether he should keep his job.
5. I never fight with anyone. When my girlfriend was upset, I agreed with whatever she said, even when I knew she was wrong, so she wouldn't leave. I don't get angry and I've never hurt myself; I just try harder to please.
6. Sister: He isn't shy at all; he'll talk to anybody and makes friends easily. He just can't start anything on his own or decide anything without someone saying it's okay. His mood settled the day he moved in with her.

What a skilled clinician weighs after each clue

1. Adjustment disorder, Major depressive disorder, Borderline personality disorder
2. Borderline personality disorder, Dependent personality disorder, Adjustment disorder
3. Borderline personality disorder, Dependent personality disorder, Avoidant personality disorder
4. Dependent personality disorder, Borderline personality disorder
5. Dependent personality disorder, Avoidant personality disorder
6. Dependent personality disorder

Sources: DSM-5-TR (APA, 2022), Personality Disorders: Dependent Personality Disorder (differential diagnosis; cultural considerations)

Patient 88: The goalie who argues

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Felix, 8, in third grade, at a behavioral health visit in his pediatrician's office. His father brought him and shares a note from his teacher.

> *"Felix: I'm not doing the stupid worksheet. Father: See? This is every single day."*

If asked about…

When did this start?	Father: At the start of second grade, a little over a year ago. That's about when his mother and I separated.
How often?	Father: There's an argument every day. Real blowups three or four times a week, at home and in class.
What happens in a blowup?	Father: He screams, stomps and slams his door. He's never hit anyone or broken anything. Twenty minutes later he wants to play cards. And it's never his fault; his sister started it, or I asked at a bad time.
Mood between blowups	Father: Happy. Silly. He tells terrible jokes at dinner. His coach says he's the best teammate on the field.
School	Father: The teacher says he argues every rule and refuses work. He hid her grade book and erased her points chart to get back at her. His grades are fine.
Friends and activities	Felix: I'm the goalie. I stopped three goals Saturday. Father: He has two good friends and plays with his cousins every weekend.
Sleep	Father: Nine or ten hours. No nightmares, no trouble falling asleep.
Medical history and medications	Father: Healthy. An inhaler for asthma once in a while. His hearing and vision were checked this year and were normal.
Safety	Father: He has never talked about hurting himself or anyone else. He's gentle with our cat.
Family history	Father: I had a temper as a kid. His mother has anxiety.
Culture and family context	Father: He lives with me during the week, and I work early shifts, so my parents help a lot. In my family, kids didn't talk back at all, so this really shocks my parents.

Only if asked (don’t volunteer)

- Got back at his teacher twice in two months
- Happy and easygoing between blowups
- Never aggressive, destructive or stealing
- Problems began around his parents' separation

Patient 88: answer and debrief

DIAGNOSIS

Oppositional defiant disorder

Daily fights, spiteful payback and blowups several times a week make DMDD and conduct disorder fair guesses. Clue 5 flips it away from DMDD: between blowups he is happy, jokes and plays well. That is the detail that separates the two: DMDD needs irritable or angry mood most of the day, nearly every day, between outbursts. Clue 6 rules out conduct disorder: no aggression to people or animals, no destruction or theft, and no serious rule violations. Losing his temper, arguing with adults, refusing rules, blaming others and getting back at his teacher twice in two months, with his father and teacher, for more than a year, meet ODD; two settings make it moderate. Exam lesson: if DMDD criteria are met, diagnose DMDD instead of ODD, so always ask about mood between outbursts. Parent management training is a first-line treatment.

What had to be ruled out

- Disruptive mood dysregulation disorder: mood between outbursts is happy, not persistently irritable
- Conduct disorder: no aggression to people or animals, property destruction, theft or serious rule violations
- ADHD: focuses when he wants to, works at grade level, no hyperactivity reported at school or soccer
- Adjustment disorder: the behavior began around his parents' separation, but full ODD criteria are met for over a year
- Intermittent explosive disorder: his outbursts are tantrums and arguments when adults set limits, typical of ODD, with no assaults; IED is added only when outbursts go beyond what ODD explains
- Normal childhood defiance: far more frequent and impairing than expected for his age, in two settings

Checkpoint: What would you ask or check NEXT?

A. Ask what his mood is like between blowups, and whether he has hurt people or animals, destroyed property or stolen **(best)**

B. Recommend a medication evaluation to control the explosive outbursts

C. Diagnose conduct disorder and refer him to a juvenile diversion program

Two questions separate the leading options: mood between outbursts (persistently irritable in DMDD, not in ODD) and serious violations of others' rights (conduct disorder). A asks both. B jumps to medication before a diagnosis is clear. C labels conduct disorder with no serious violation on record, and diversion programs are for youth involved with the courts.

The case in six clues (as the app reveals it)

1. Father: Anything I ask, homework, bath, shoes, turns into a fight. He yells 'You can't make me' and slams his door, and afterward it's always someone else's fault: his sister started it, or I asked at a bad time.
2. Teacher's note: Felix argues with me about every rule and refuses to work when I ask. Twice in the past two months he has 'gotten back' at me: he hid my grade book in the coat closet, and he erased our class points chart after I kept him in from recess.
3. Father: The blowups happen three or four times a week, at home and at school. He screams and stomps, and it can take twenty minutes for him to settle.
4. Father: This has been going on since the start of second grade, more than a year now. It happens with me and with his teacher; his mother says he's easier at her place on weekends.
5. Father: Between blowups he's a happy kid. He jokes at dinner, plays goalie on his soccer team, and his coach says he's a pleasure. Once a fight is over, he's over it.
6. Teacher's note: He has never hit or threatened anyone, never broken or stolen anything, and never left class without permission. His reading and math are on grade level, and he can sit and focus when he wants to.

What a skilled clinician weighs after each clue

1. Oppositional defiant disorder, Disruptive mood dysregulation disorder, ADHD
2. Oppositional defiant disorder, Conduct disorder, Disruptive mood dysregulation disorder
3. Disruptive mood dysregulation disorder, Oppositional defiant disorder, Conduct disorder
4. Disruptive mood dysregulation disorder, Oppositional defiant disorder
5. Oppositional defiant disorder, Conduct disorder
6. Oppositional defiant disorder

Sources: DSM-5-TR (APA, 2022), Disruptive, Impulse-Control, and Conduct Disorders: Oppositional Defiant Disorder; Kaminski & Claussen (2017), Evidence base update for psychosocial treatments for disruptive behaviors in children, Journal of Clinical Child and Adolescent Psychology 46(4):477-499

Patient 89: The dentist's referral

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Imani, 27, a preschool teacher, at a behavioral health visit in her community health center. Her dentist in the same building referred her and sent a note, and the clinic nurse joins.

> *"My dentist sent me over. I'm honestly not sure why I'm here."*

If asked about…

When did this start?	About a year and a half ago, after a breakup. I started dieting hard, and then the binges started.
A typical day	Black coffee, a five-mile run before work, nothing until a small dinner. Then two or three nights a week I lose it.
The binges	Cookies, chips, leftovers, whatever's there, standing at the counter, fast, until I feel sick. Always alone. I can't stop once it starts.
After a binge	I make myself throw up. Sometimes I run extra the next morning. I've never used laxatives or diet pills.
Weight and body image	My weight hasn't really changed in two years, but in my head I'm always about to get fat. A good day is a day I feel thin.
Medical history	My teeth, my swollen jaw, and now the potassium. I get dizzy when I stand up fast. My periods are regular. No heart problems that I know of.
Mood and thoughts of suicide	I feel ashamed after a binge, but I'm not depressed most days. I've never hurt myself, and I don't want to die.
Alcohol and drugs	Wine on weekends with friends. No drugs. No medicines.
Family history	My mom has always dieted. My aunt was treated for an eating disorder in college.
Work and social life	I'm good at my job, and the kids love me. But I skip staff dinners so nobody sees me eat.
Culture and context	In my family, people comment on bodies all the time, and losing weight gets you compliments. Nobody expects a Black woman at a normal weight to have an eating disorder. My last doctor never asked.

Only if asked (don’t volunteer)

- Vomits after binges two or three times a week
- The pattern has lasted about a year and a half
- Dizzy on standing; potassium is low
- An aunt was treated for an eating disorder

Patient 89: answer and debrief

DIAGNOSIS

Bulimia nervosa

Strict restriction, daily runs and self-worth tied to weight make anorexia the first guess, and the binges add binge-eating disorder. Clue 4 shows regular self-induced vomiting, which rules out binge-eating disorder. Clue 5 flips it from anorexia: her weight is in the healthy range and steady. That is the detail that separates the two: anorexia requires significantly low body weight, so binge-purge behavior at a low weight is anorexia, binge-eating/purging type, while the same behavior at a normal weight is bulimia. Binges, each followed by vomiting, two to three times a week for a year and a half, with self-worth driven by shape and weight, meet bulimia nervosa. The vomiting alone is well above the once-a-week threshold; her routine morning run counts as compensatory only when it is used to make up for a binge or is excessive, so the call does not depend on it. Exam lesson: bulimia often hides at a normal weight, and dentists may spot it first. Purging can drop potassium and cause dangerous heart rhythms, so medical monitoring goes with therapy; CBT-E is first-line.

What had to be ruled out

- Anorexia nervosa, binge-eating/purging type: her weight is healthy and stable
- Binge-eating disorder: she vomits after every binge, two or three times a week
- Avoidant/restrictive food intake disorder: her restriction is driven by weight and shape concerns
- A medical cause of vomiting: the vomiting is self-induced after binges

Checkpoint: What would you ask or check NEXT?

A. Start weekly therapy for her eating right away and check on her medical status in a month or two

B. Advise her to stop the vomiting by keeping binge foods out of the house and eating less overall

C. Ask her doctor to check her weight, vital signs and electrolytes, and ask how often the binges and vomiting happen **(best)**

Self-induced vomiting can lower potassium and cause dangerous heart rhythms, so a medical check comes first, and weight and frequency are what separate bulimia from anorexia and set severity. A delays the medical safety check. B is harmful advice: restricting intake drives the binge-purge cycle, and treatment works toward regular eating.

The case in six clues (as the app reveals it)

1. I think about food and my body all day. Most days I'm really strict: black coffee until dinner and a run every morning.
2. If I gain even two pounds, I feel like a failure at everything. How I look decides whether it's a good day.
3. A few nights a week I lose control. I'll eat a sleeve of cookies, a family-size bag of chips and the leftover pasta standing at the counter, and I can't stop.
4. Dentist's note: She has enamel wear on the backs of her front teeth and swollen glands along her jaw. She admitted she makes herself throw up after the big nights.
5. Nurse: Her weight is in the middle of the healthy range for her height, and her records show it has been steady for two years.
6. Nurse: She says she makes herself vomit after every binge, two or three times a week, and has for about a year and a half. Her potassium came back a little low, so her doctor wants to see her this week.

What a skilled clinician weighs after each clue

1. Anorexia nervosa, Bulimia nervosa, Binge-eating disorder
2. Anorexia nervosa, Bulimia nervosa
3. Bulimia nervosa, Binge-eating disorder, Anorexia nervosa
4. Bulimia nervosa, Anorexia nervosa
5. Bulimia nervosa
6. Bulimia nervosa

Sources: DSM-5-TR (APA, 2022), Feeding and Eating Disorders: Bulimia Nervosa; American Psychiatric Association (2023), Practice Guideline for the Treatment of Patients With Eating Disorders, 4th ed., Medical assessment and CBT for bulimia nervosa

Patient 90: The drive-thru on the way home

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Hector, 52, an insurance agent, at a pre-surgery evaluation with the social worker in a hospital weight-loss surgery program. His wife, Lourdes, joins for part of the visit.

> *"My surgeon said I have to talk to you before they'll schedule me. I'll be honest: I've got a problem with food."*

If asked about…

When did this start?	Off and on since my twenties. It's been bad, two or three times a week, for at least a year, since work got more stressful.
What a binge looks like	Drive-thru on the way home, or a large pizza and a pint of ice cream, eaten in the car in the parking lot. Fast, until I feel sick. Then I go in and eat dinner with the family.
Making up for it	I tried throwing up once, two years ago, and couldn't. No laxatives, no water pills. I don't skip meals and I don't hit the gym to burn it off. I just promise myself I'll diet.
Dieting history	Every diet there is. I last about two weeks.
Mood	Ashamed after a binge. Otherwise I'm good. I like my life. Wife: He's an upbeat guy, except about this.
Sleep	Wife: He snores, and his doctor ordered a sleep study. He sleeps about seven hours.
Alcohol, drugs, medications	A beer at ball games. No drugs. Metformin for blood sugar and a blood pressure pill.
Medical history	Type 2 diabetes for three years, high blood pressure, and bad knees. That's why the surgeon thinks surgery could help.
Thoughts of suicide or harming others	No, never. I feel bad about the eating, but I've never thought about hurting myself or anyone.
Family history	My mother was an emotional eater. My father had diabetes.
Work and functioning	I'm one of the top agents in my office, and I coach Little League. Sometimes I'm late to practice because I was eating in the car.
Culture and context	Sunday dinner at my mother's is a big deal, and turning down seconds feels like an insult. But that's not the problem. The binges happen alone, in the car, where nobody sees.

Only if asked (don’t volunteer)

- Tried to vomit once, two years ago, and never again
- No laxatives, fasting or compensatory exercise
- Binges two or three times a week for at least a year
- Eats in the car before family dinner

Patient 90: answer and debrief

DIAGNOSIS

Binge-eating disorder

Secret eating, disgust, talk of 'making up for it' and an attempt to vomit make bulimia a fair early guess. Clue 4 flips it: he tried to vomit once, two years ago, and has no regular compensatory behavior at all, no purging, laxatives, fasting or exercise. That is the detail that separates the two: bulimia needs compensatory behavior about once a week for 3 months along with the binges, and binge-eating disorder has none. Binges two to three times a week for over a year, with loss of control, eating fast, eating until he feels sick, eating a full dinner right after, eating alone, and marked distress, meet binge-eating disorder, mild by binge frequency. His mood is good apart from the binges, so depression does not explain it. Exam lesson: binge-eating disorder is the most common eating disorder in US adults and occurs at any weight and in men. CBT is first-line, and lisdexamfetamine is FDA-approved for moderate to severe cases. Binge eating should be treated and followed before and after weight-loss surgery, because loss-of-control eating can return afterward.

What had to be ruled out

- Bulimia nervosa: no regular compensatory behavior; one failed attempt to vomit two years ago
- Anorexia nervosa: no restriction and no low weight
- Major depressive disorder: mood is good outside the binges, and he still enjoys coaching and work
- No mental disorder (ordinary overeating): loss of control, frequency and marked distress go well beyond occasional overeating

Checkpoint: What would you ask or check NEXT?

A. Tell him the program cannot schedule surgery until he stops binge eating completely

B. Ask what 'making up for it' means and how often, if ever, he vomits, uses laxatives, fasts or exercises to compensate **(best)**

C. Diagnose bulimia nervosa and refer him to a residential eating disorder program

The frequency of compensatory behavior is what separates bulimia from binge-eating disorder, so it has to be asked about directly; an isolated attempt would not count. A makes a surgical decision that is not the social worker's to make, before any assessment. C diagnoses without knowing the frequency, and residential care is far beyond what the facts so far support.

The case in six clues (as the app reveals it)

1. Food has been running my life. I eat in secret, and afterward I feel disgusting.
2. After a bad night I tell myself I'll make up for it. I've tried making myself throw up after a binge.
3. Wife: I find wrappers hidden in his car two or three times a week. The one time I caught him, he was eating so fast he barely tasted it, and then he came inside and ate a full dinner with us.
4. I tried throwing up exactly once, about two years ago, and couldn't do it. I don't vomit, I've never used laxatives or water pills, I don't skip meals and I don't exercise to make up for it. 'Making up for it' just means promising myself a diet that never starts.
5. A binge is a whole large pizza and a pint of ice cream in under an hour, alone in the car, until I feel sick. I can't stop once I start, and it's been two or three times a week for at least a year.
6. Wife: He hates himself afterward and has cried about it. Otherwise he's in good spirits; he coaches our son's Little League team and loves his job.

What a skilled clinician weighs after each clue

1. Binge-eating disorder, Bulimia nervosa, Major depressive disorder
2. Bulimia nervosa, Binge-eating disorder
3. Bulimia nervosa, Binge-eating disorder
4. Binge-eating disorder, Bulimia nervosa
5. Binge-eating disorder, Major depressive disorder
6. Binge-eating disorder

Sources: DSM-5-TR (APA, 2022), Feeding and Eating Disorders: Binge-Eating Disorder; American Psychiatric Association (2023), Practice Guideline for the Treatment of Patients With Eating Disorders, 4th ed., Psychotherapy and medication for binge-eating disorder